

Morpheaform Dermal Elastosis (MDE)

An inflammatory phase, consisting of indurated plaques or urticaria, malaise, and fever, precedes the development of diffuse wrinkling, atrophy, and severe disfigurement. Insect bites may trigger the initial inflammatory lesions.

TREATMENT

There is no known effective treatment for MDE. Sunscreens, colchicine, and topical retinoic acid have been tried without significant success.

Striae (See Chaps. 107 and 152)

Striae are very common and typically develop between the ages of 5 and 50 years. They occur approximately twice as frequently in women as in men. Common periods of onset include puberty, with an incidence of 25% to 35%, and pregnancy, where the incidence rises to 77%.

The exact pathogenesis of striae is not fully understood. **Striae distensae** result from tears in the connective tissue leading to dermal atrophy. Contributing factors include hormonal influences (particularly corticosteroids), mechanical stress, and genetic predisposition.

During puberty, striae commonly appear in areas of rapid growth. In girls, typical sites include the breasts, thighs, hips, and buttocks. In boys, they are most often seen on the shoulders, lumbosacral region, and thighs. Less common locations include the abdomen, upper arms, neck, and axillae.

In pregnancy, striae are frequently observed on the abdomen, and less commonly on the breasts and thighs, especially during the third trimester. They are more prevalent in younger primigravidas. **Striae gravidarum** may be associated with an increased risk of perineal lacerations during vaginal delivery.

CLINICAL FINDINGS

Striae are typically multiple, symmetric, well-defined linear atrophic lesions that follow Langer's lines of skin tension. Initially, they present as red-to-violaceous, slightly elevated lines known as **striae rubra**. Over time, the color fades, and the lesions become atrophic, developing a fine, white, wrinkled appearance referred to as **striae alba**. Lesions can range from several centimeters in length and a few millimeters to several centimeters in width. Those associated with systemic corticosteroid use or Cushing syndrome tend to be larger and more widespread.

HISTOPATHOLOGY

Histologically, striae show reduced dermal thickness and diminished collagen in the upper dermis. Collagen bundles are thinned and run parallel to the epidermis, with some oriented transversely to the long axis of the striae. Elastic fiber changes are variable; elastin may be fragmented, and specific staining reveals a marked reduction in elastin content compared to adjacent normal skin. Hair follicles and other skin appendages are absent within the lesions.

DIFFERENTIAL DIAGNOSIS

The diagnosis of striae distensae is usually clinical and straightforward. However, **linear focal elastosis** (also known as elastic striae), first described by Burkett et al. in 1989, should be considered. This condition presents as yellow, palpable, striae-like bands, typically on the lower back. Unlike striae alba, these lesions are raised rather than depressed and have a yellow hue. It predominantly affects elderly men, though cases in adolescents have been reported. Histologically, linear focal elastosis shows a focal increase in elongated or fragmented elastic fibers and dermal thickening. It is hypothesized to represent an overactive regenerative response of elastic tissue and may be considered a keloidal repair process following striae distensae.

TREATMENT

Striae distensae have no medical consequences but are often a source of cosmetic concern. Since stretch marks may fade spontaneously over time, the efficacy of many proposed treatments—especially those lacking controlled studies—is difficult to evaluate.